

Management of Unipolar Major Depression and Generalized Anxiety Disorder

Primary Care Essentials

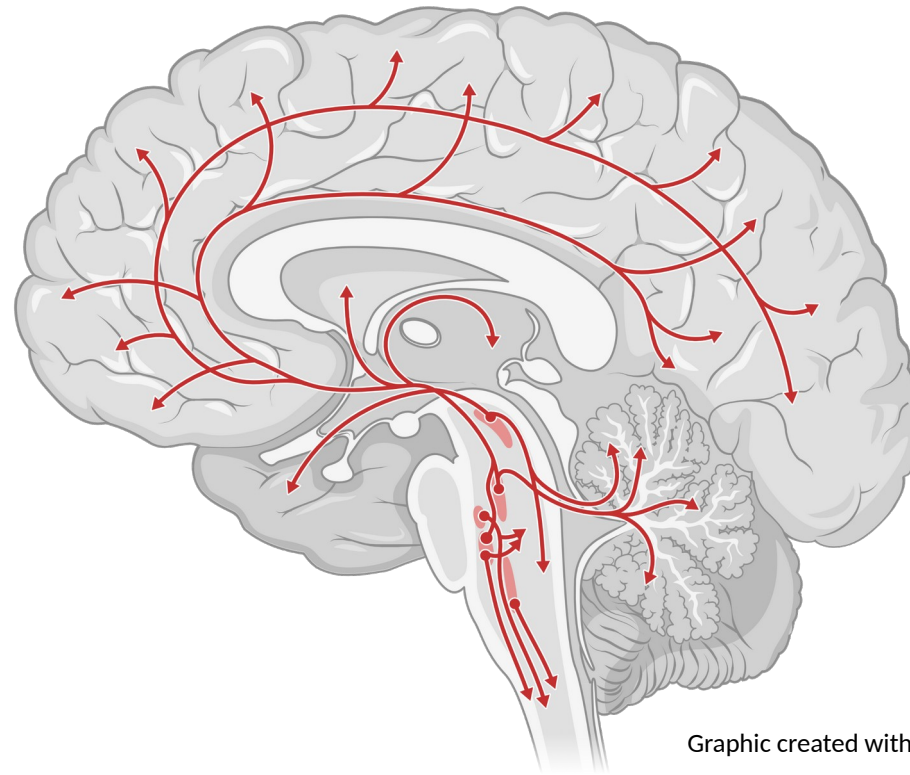
Screening &
Diagnosis

Differential

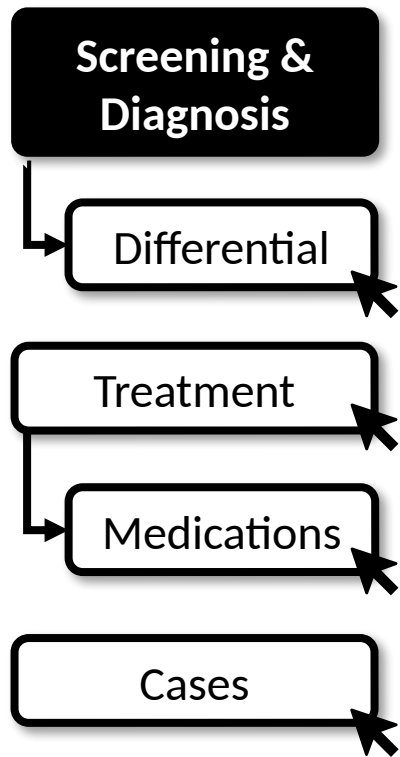
Treatment

Medications

Cases



Graphic created with BioRender.com



Why screen?

1. Screening doubles detection of depression.
2. Treatment of depression -> Improves **quality of life**, reduces **mortality**, and reduces **economic burden**

Screening and
Diagnosis Tools

Unipolar Major
Depressive Disorder

Low risk screen

Generalized Anxiety
Disorder

Diagnosis: Must cause impairment of social, occupational, or other important areas of functioning. Score defines severity.

PHQ-9		In the past two weeks have you felt:		Not at all	Several days	More than half the days	Nearly every day
1.	Little interest or pleasure in doing things			0	1	2	3
2.	Feeling down, depressed, or hopeless			0	1	2	3
3.	Trouble falling or staying asleep, or sleeping too much			0	1	2	3
4.	Feeling tired or having little energy			0	1	2	3
5.	Poor appetite or overeating			0	1	2	3
6.	Feeling bad about yourself — or that you are a failure or have let yourself or your family down			0	1	2	3
7.	Trouble concentrating on things, such as reading the newspaper or watching television			0	1	2	3
8.	Moving or speaking so slowly that other people could have noticed? Or the opposite — being so fidgety or restless that you have been moving around a lot more than usual			0	1	2	3
9.	Thoughts that you would be better off dead or of hurting yourself in some way			0	1	2	3

Screening & Diagnosis

Differential

Treatment

Medications

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Why screen?

Screening and
Diagnosis Tools

Unipolar Major
Depressive Disorder

Generalized Anxiety
Disorder

Low risk screen?

Describe the process of
screening and diagnosing
in your clinic?

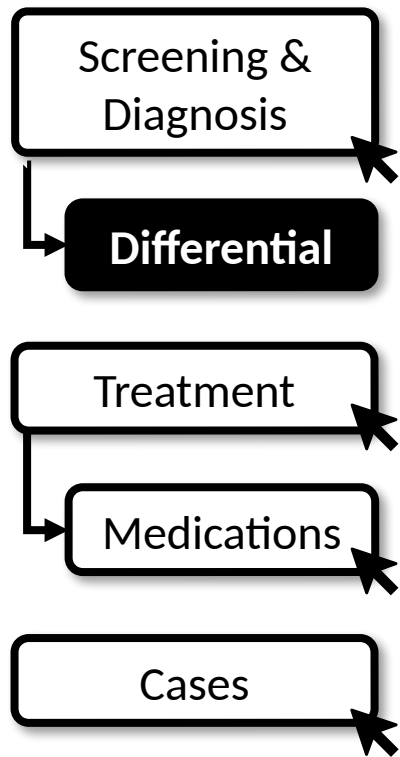
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2. Treatment of depression -> Improves **quality of life**, reduces **mortality**, and reduces **economic burden**

GAD-7

Diagnosis: 6+ months of excessive and difficult to control anxiety with **3+ symptoms**

Over the <u>last 2 weeks</u> , how often have you been bothered by the following problems? (Use "✓" to indicate your answer)	Not at all	Several days	More than half the days	Nearly every day
1. Feeling nervous, anxious or on edge	0	1	2	3
2. Not being able to stop or control worrying	0	1	2	3
3. Worrying too much about different things	0	1	2	3
4. Trouble relaxing	0	1	2	3
5. Being so restless that it is hard to sit still	0	1	2	3
6. Becoming easily annoyed or irritable	0	1	2	3
7. Feeling afraid as if something awful might happen	0	1	2	3

(For office coding: Total Score T ____ = ____ + ____ + ____)



Labs to consider?



CBC



HCG



BMP



TSH

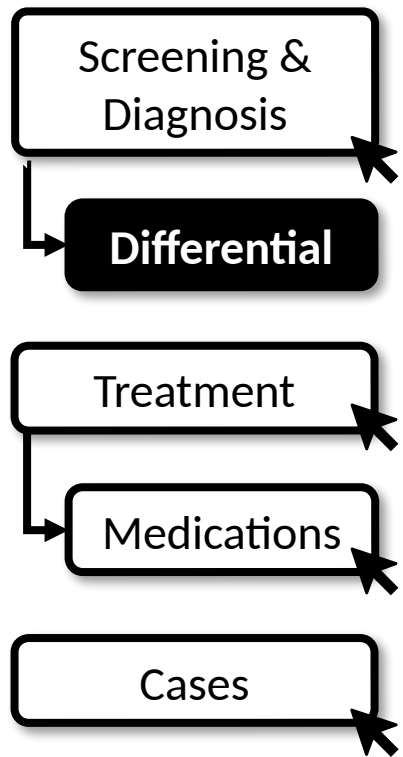
Unipolar Major Depressive Disorder

Generalized Anxiety Disorder

- Adjustment Disorder with Depressed Mood
- ADHD
- ***Bipolar disorder**
- Borderline Personality disorder
- Complicated Grief
- Delirium
- Hypothyroidism
- Schizophrenia and Schizoaffective Disorder
- Substance Use Disorders

Clues that raise suspicion of Bipolar Disease?

1. Age < 25
2. Family History of Bipolar Disorder
3. Multiple (at least 3-5) recurrences of Major Depression
4. What screening questions would you ask?



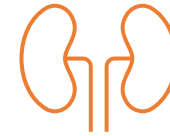
Labs to consider?



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TSH

Unipolar Major
Depressive Disorder

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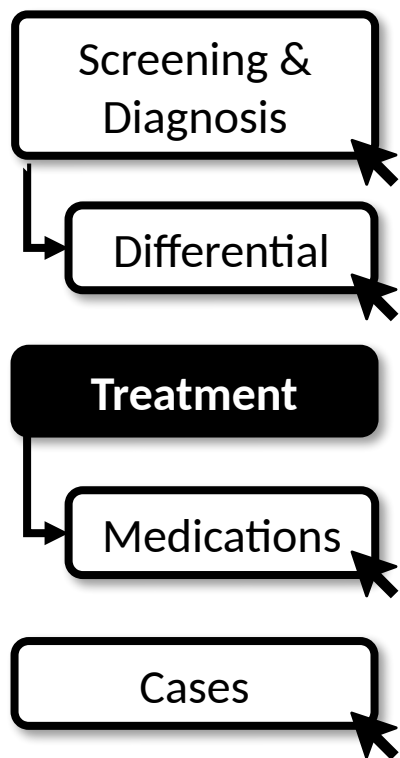
Panic Disorder

Adjustment Disorder

Obsessive-Compulsive Disorder

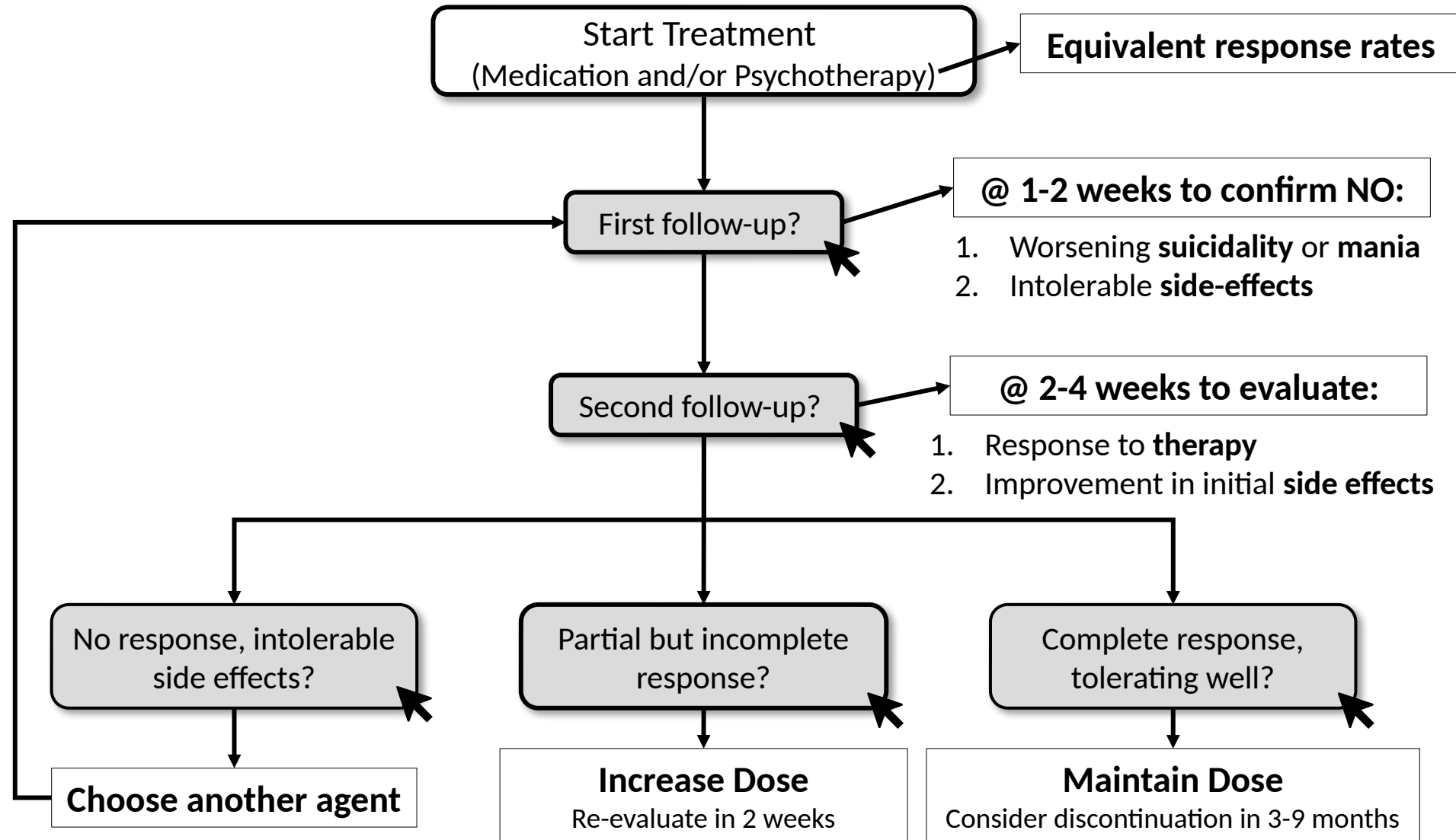
Separation Anxiety Disorder

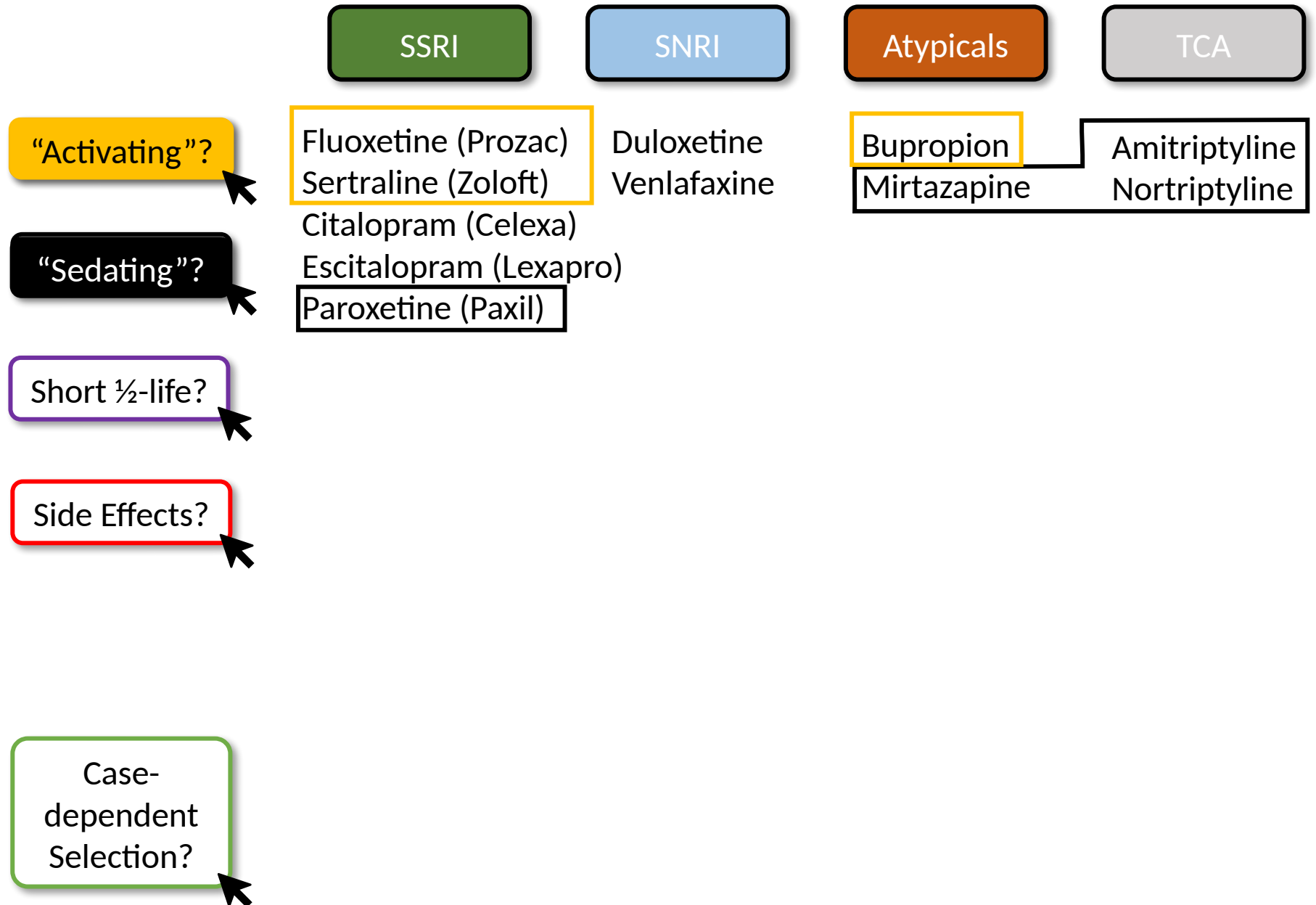
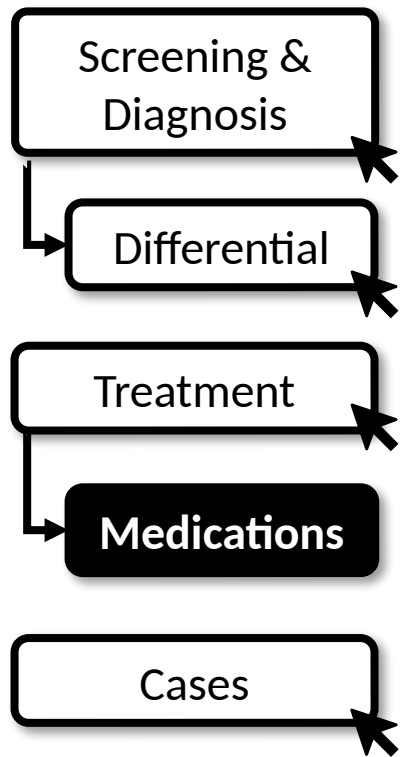
Post-Traumatic Stress Disorder

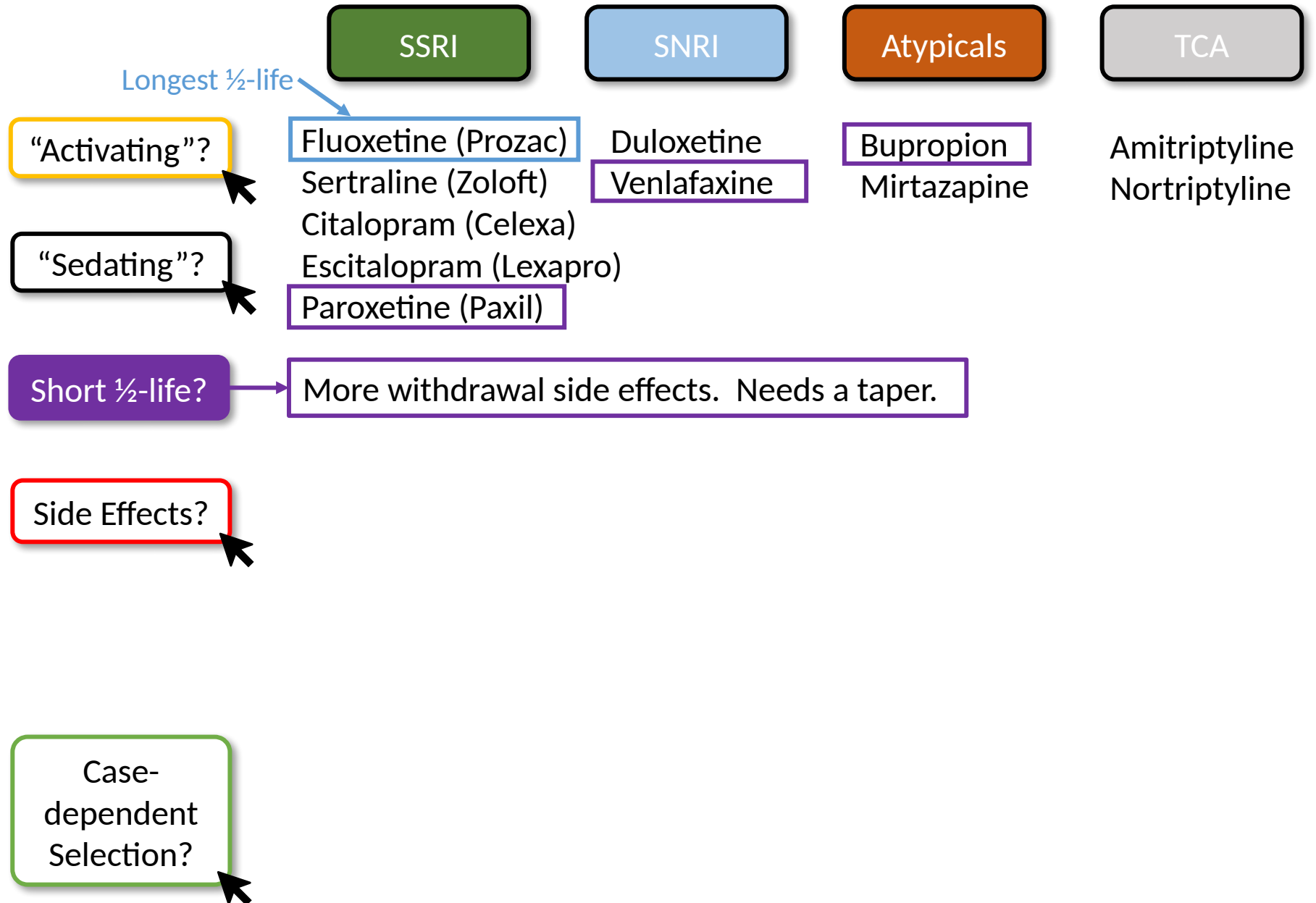
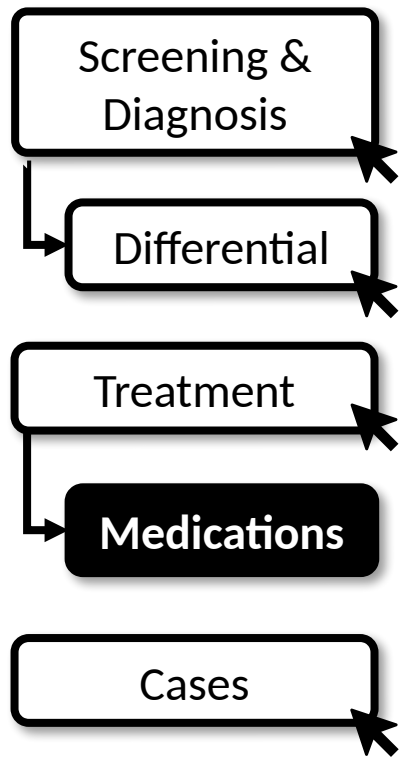


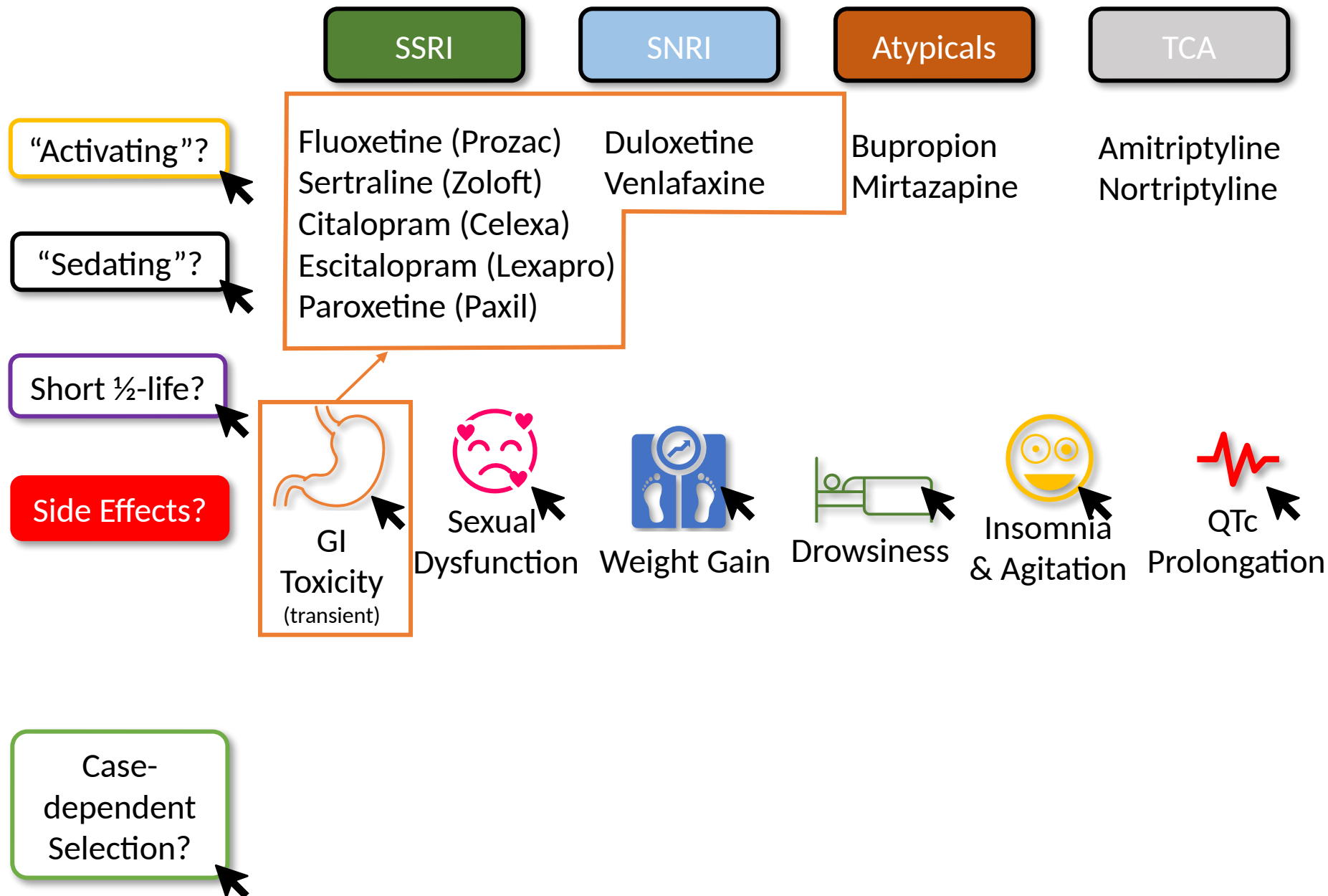
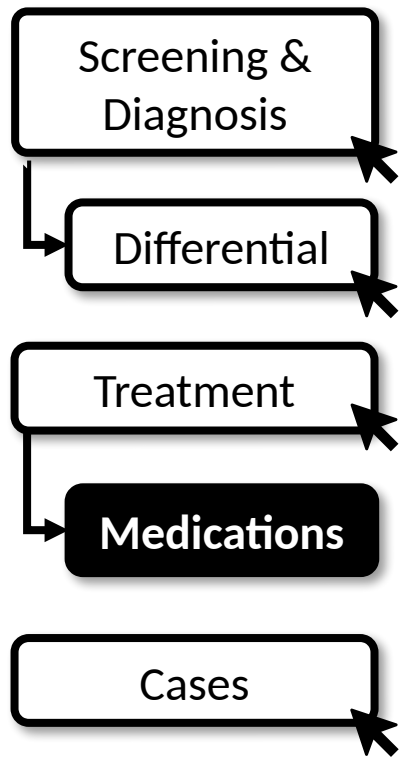
Goals of Depression Treatment

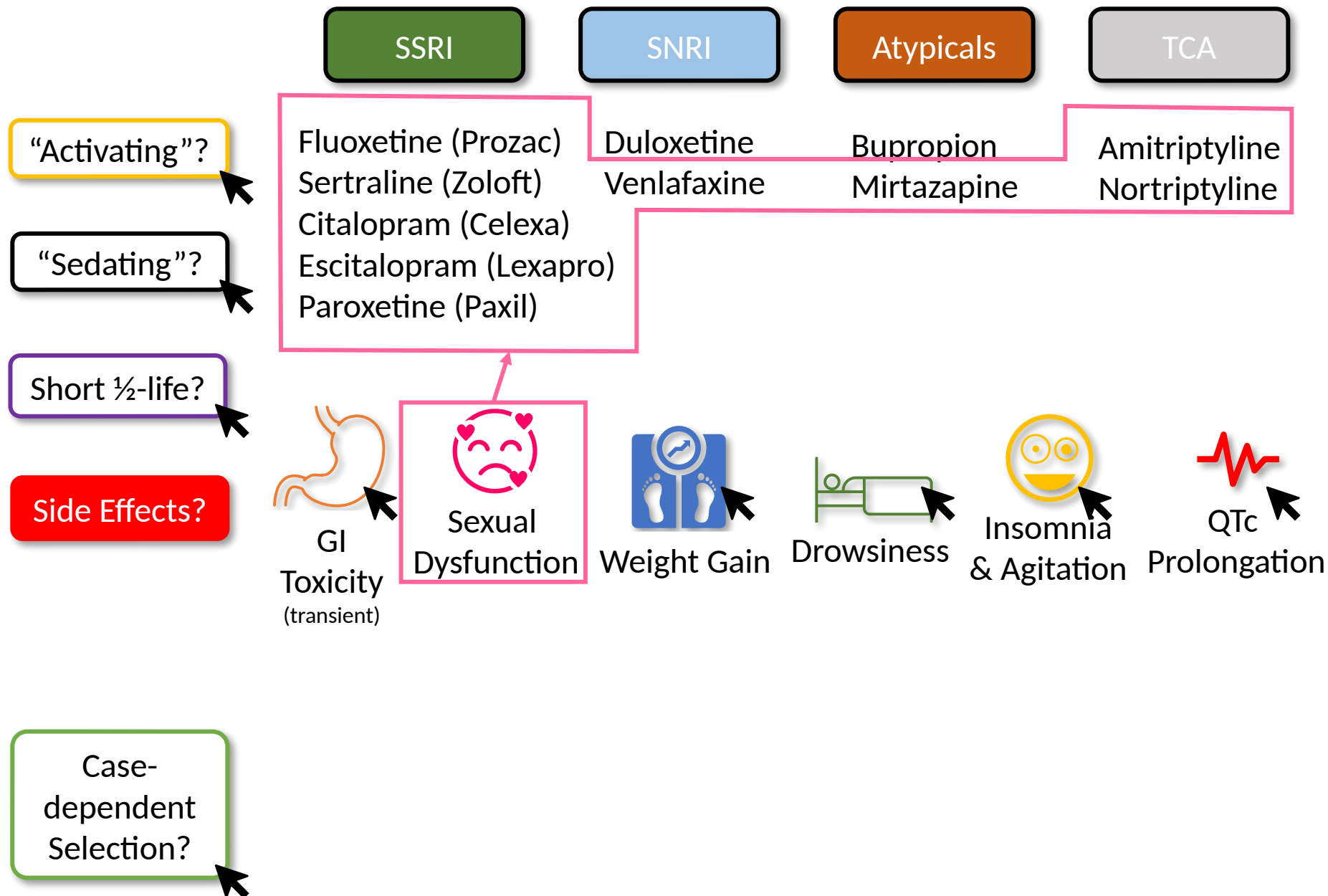
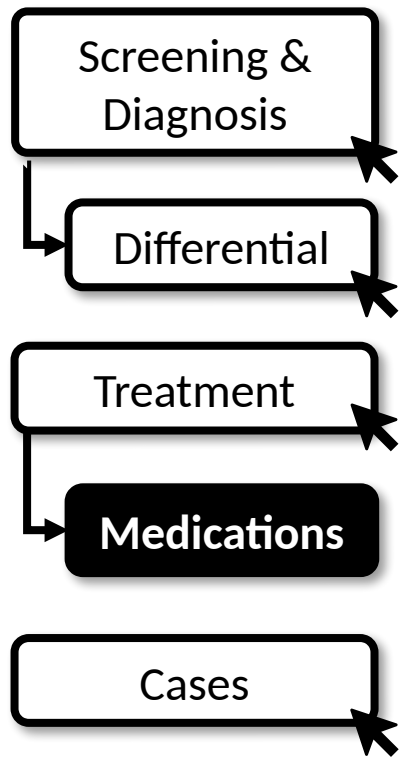
Patient focused: What are the patient's goals for therapy?
Remission = Absence of sad mood and reduced interest x 3 weeks

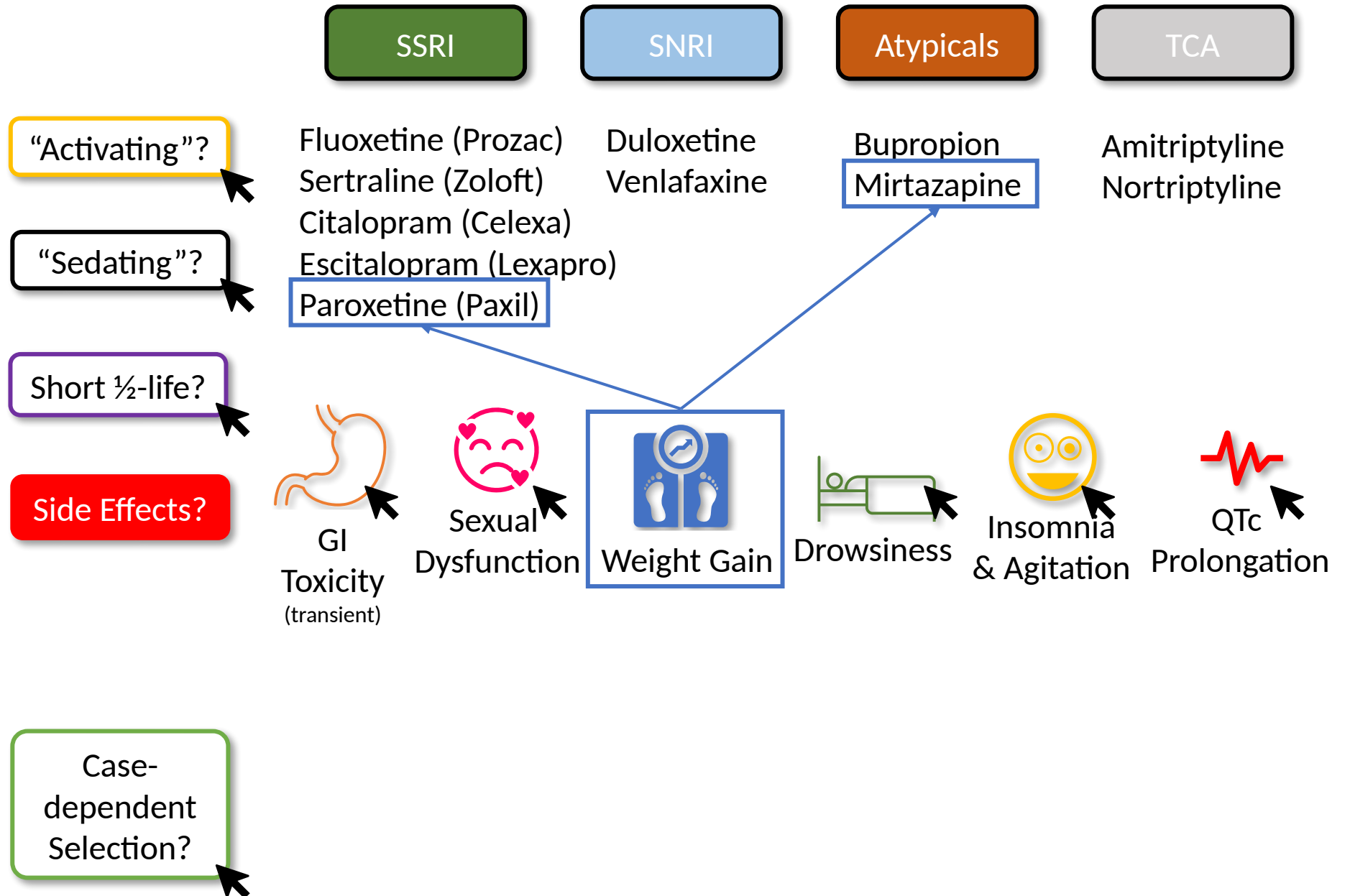
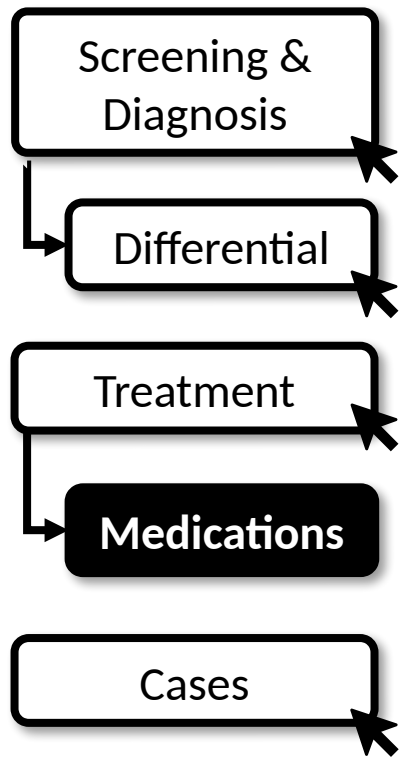


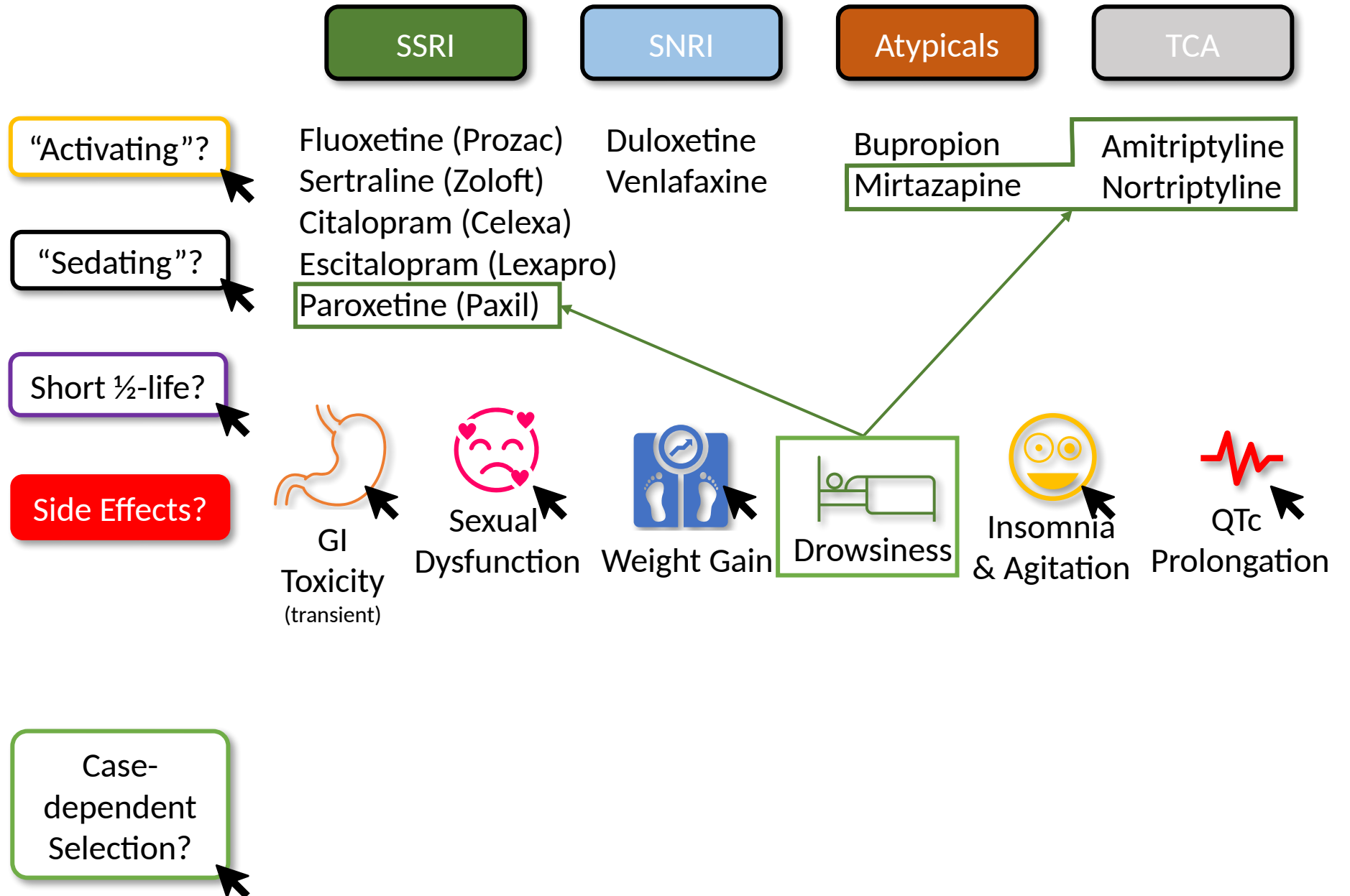
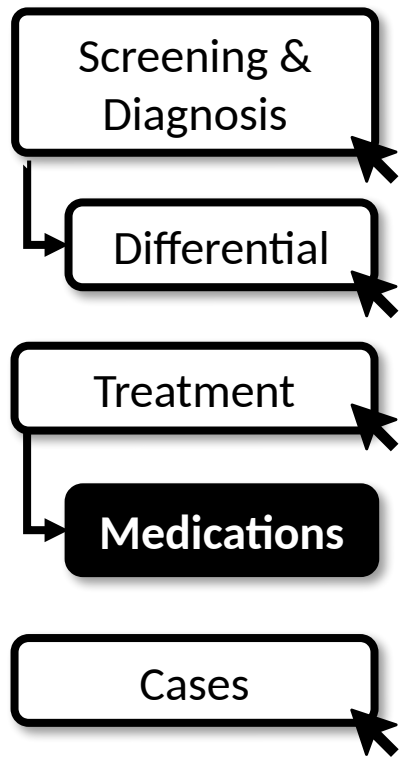


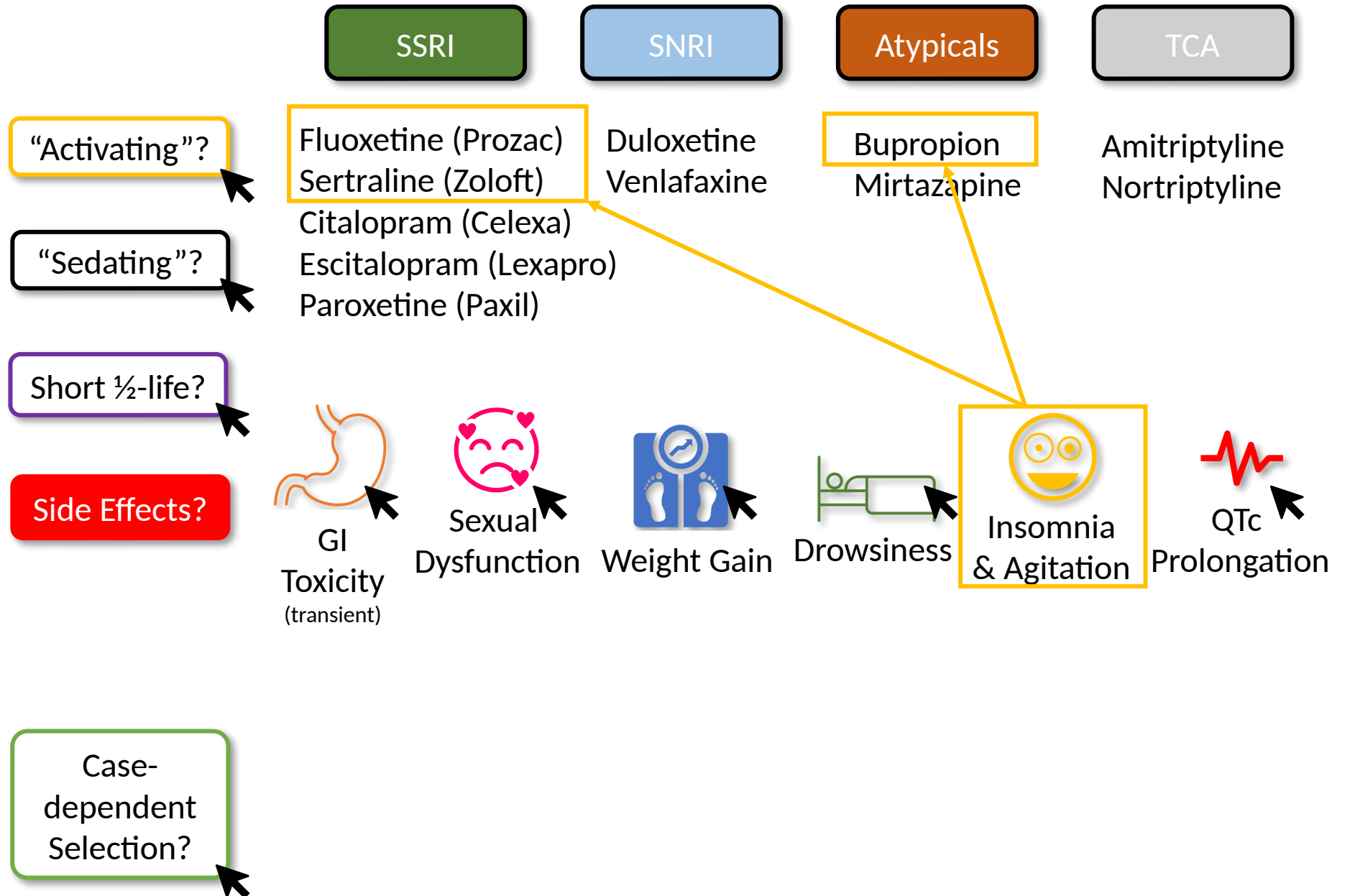
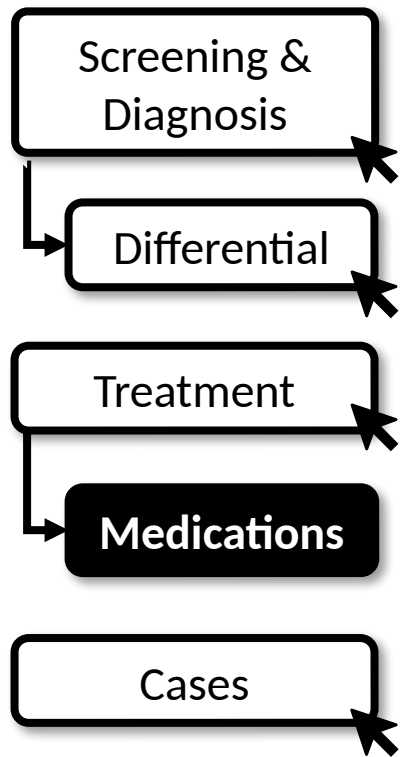


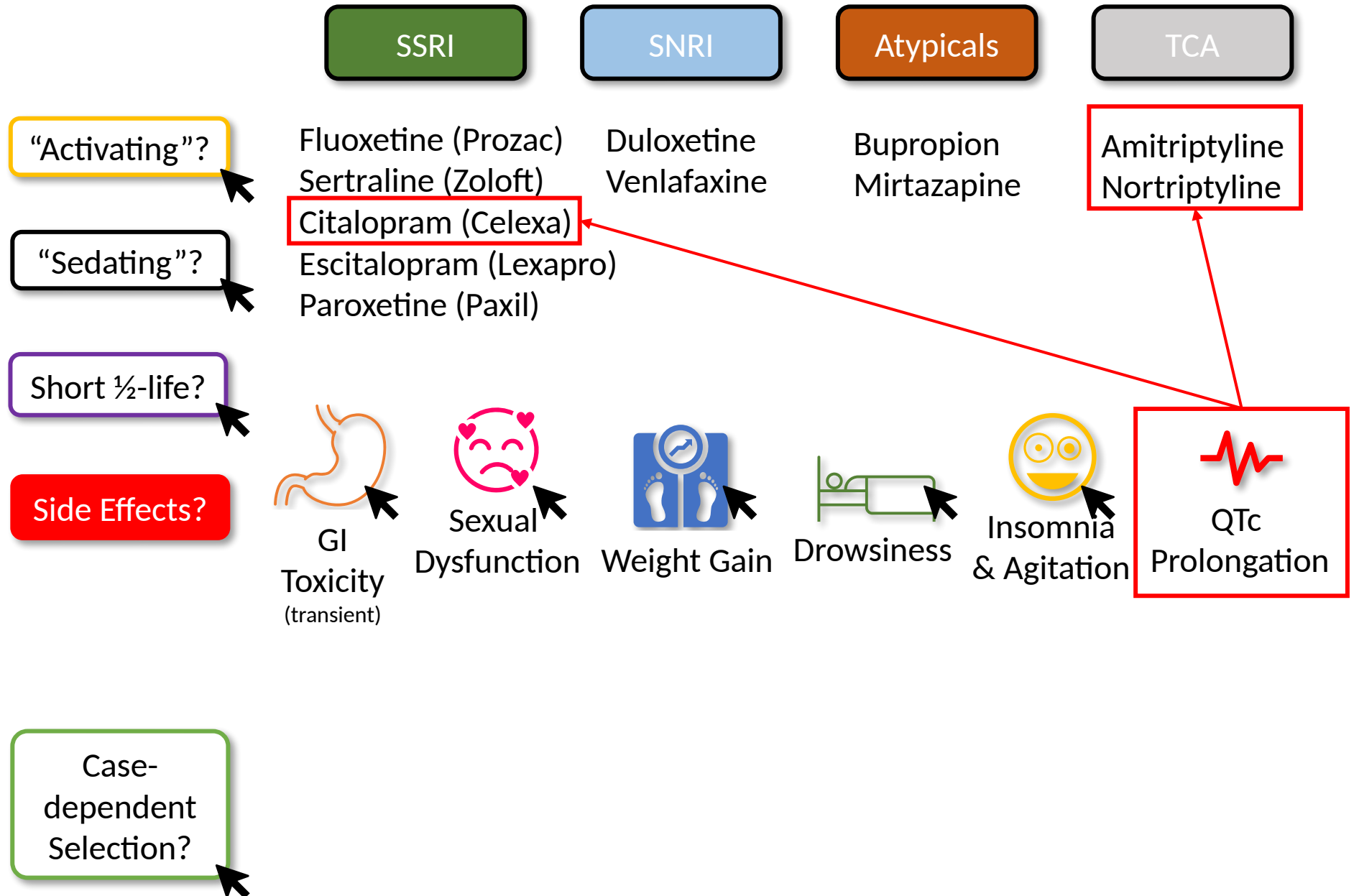
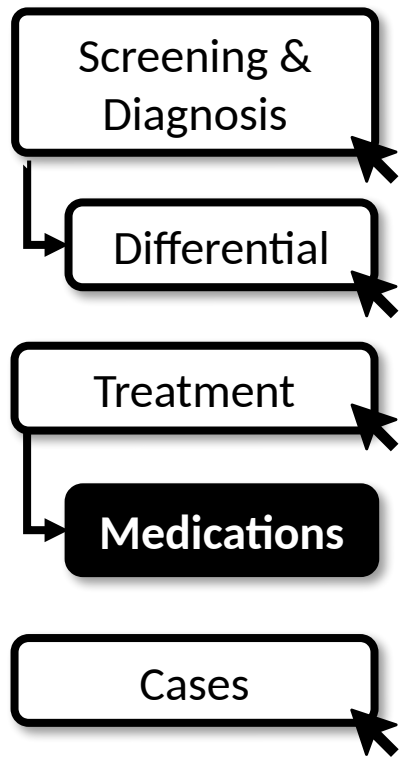


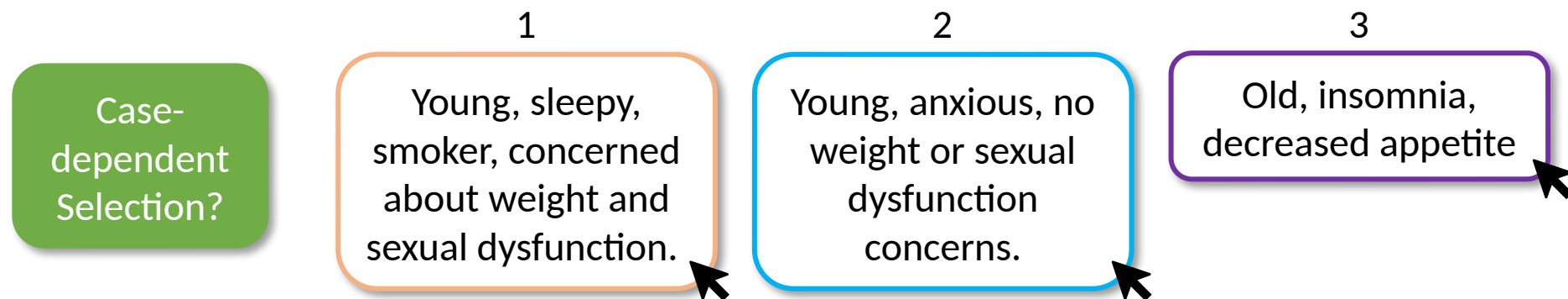
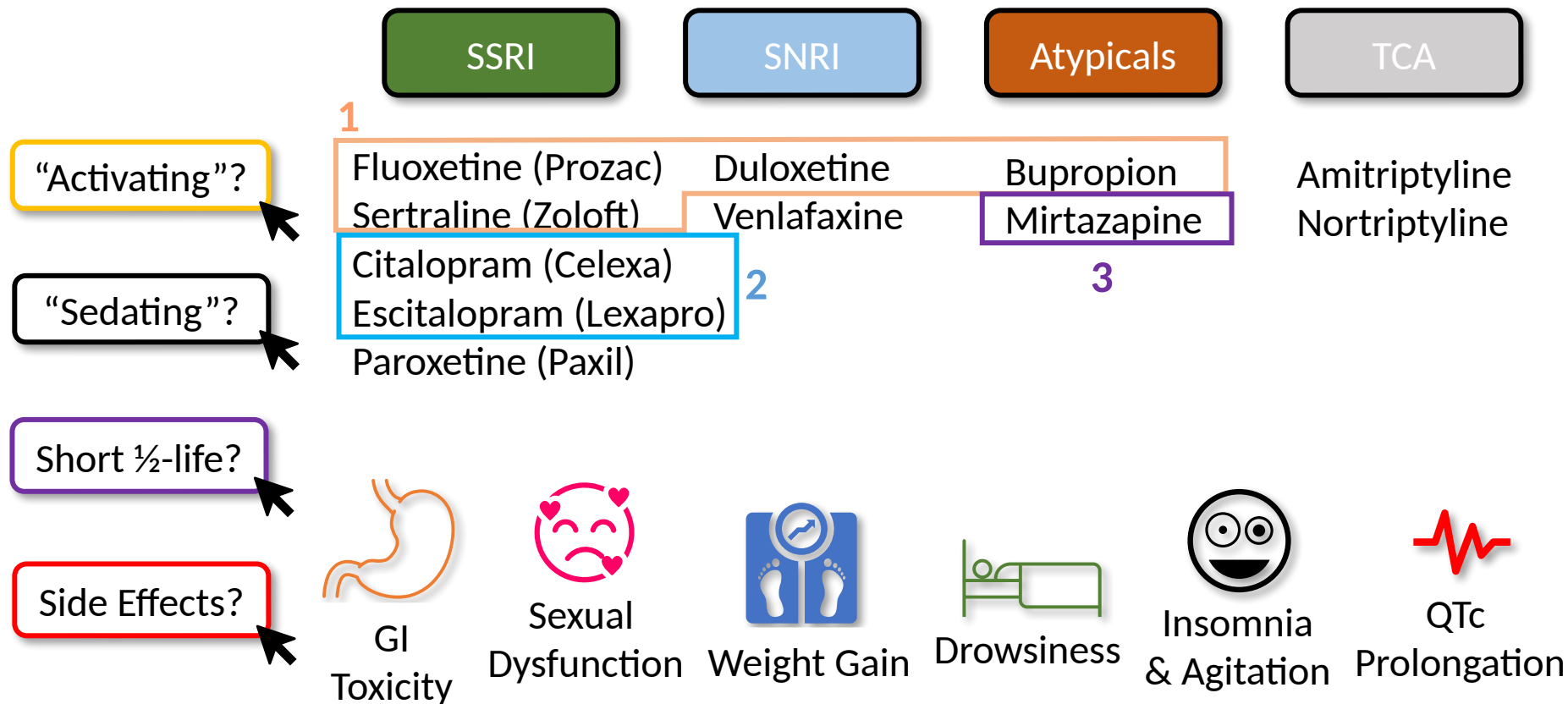
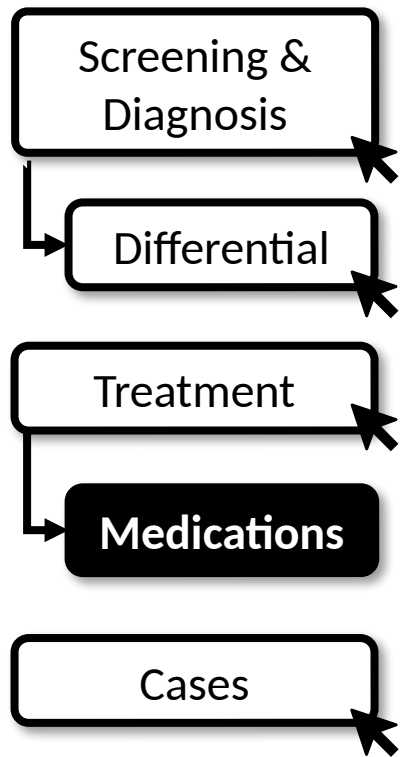












Case 1

25-year-old man presents to establish care with his primary care doctor. He reports no past medical history and otherwise feels well. He moved from Chicago 2 months ago and has noticed increasing feelings of loneliness and sadness over the past several weeks. He also reports increasing stress with his new job

Which screening test is most appropriate?

PHQ-9 ([seen here](#)), also consider GAD-7

After further questioning, patient meets criteria for Moderate Depression, what is the next step?

Suicide Risk Assessment

Patient denies active suicidal ideation and has no prior history of suicide attempts. What medication would you recommend?

SSRI or SNRI, consider avoiding Paroxetine

When should this patient return for follow up?

1-2 weeks for side effect evaluation and dosage increase

Case 2

79-year-old woman presents to her geriatrician for evaluation of fatigue and poor concentration. She has a PMH of HTN, RA, and CAD. Her symptoms started approximately 6 months ago after a MI. Her follow up with her cardiologist has been normal. She denies chest pain, dizziness, lightheadedness.

VS: Ht 5'10", Wt 130lbs (previously 146), HR 82, BP 138/80, RR 16, SpO2 95%

Exam: Unremarkable cardiac, neuro and pulmonary exams.

Meds: Carvedilol, Losartan, MTX, ASA, Atorvastatin

What screening labs and questionnaires would be appropriate?

EKG, CBC, BMP, TSH. PHQ-9 or GDS-5, MOCA

Her labs, ECG, and MOCA are unremarkable and her GDS-5 is positive. What treatment regimen would you recommend?

Citalopram or Mirtazapine, CBT

Patient starts Citalopram (10mg) and returns in 2 weeks. She reports mild diarrhea that has since improved. Her GDS-5 is unchanged. Next step?

Increase Citalopram to 20mg

Case 3



28-year-old woman presents with general arthralgias and low back pain that started several years ago. She feels "restless" and "on edge", which has been going on since she was in her late teens. She states she has had intermittent depression but has never sought treatment as these episodes resolved on their own. She drinks 2 glasses of wine each night to help with sleep.

VS: Ht 5'3", Wt 125 lbs, HR 72, BP 120/80, RR 16, SpO2 95%

PE: Unremarkable MSK exam

Which screening test is most appropriate?

GAD-7, AUDIT/CAGE, PHQ-9

You diagnose her with Unipolar Depression, and Generalized Anxiety Disorder. What do you offer for treatment?

Physical Therapy, CBT, Duloxetine

On 8 week follow up, she is on a max dose of duloxetine and her GAD7 has improved but is still showing moderate anxiety. Next step?

Switch to different SRI, or add Pregabalin/Buspirone

She switches to sertraline and on follow-up 2 weeks later, reports feeling "better than ever". Her energy is at an "all time high" and her pain is "entirely gone"

Reassess Diagnosis

Assessing Suicide
Risk



Critical step at each visit while treating patients for depression and anxiety. Use Clear language. Final question in PHQ-9.

Next steps



Describe the next steps in managing a patient with active suicidal ideation in your clinic.

- Where is your safety plan handouts?
- What mental health resources are immediately available?

Additional
Assessment Tools



National Institute of Mental Health



Short (5 questions)
Needs further questioning
about safety if positive

Columbia Suicide Severity Rating Scale



Very long and
detailed.

BACK

		Not at all	Several days	More than half the days	Nearly every day
1.	Little interest or pleasure in doing things	0	1	2	3
2.	Feeling down, depressed, or hopeless	0	1	2	3
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4.	Feeling tired or having little energy	0	1	2	3
5.	Poor appetite or overeating	0	1	2	3
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7.	Trouble concentrating on things, such as reading the newspaper or watching television	0	1	2	3
8.	Moving or speaking so slowly that other people could have noticed? Or the opposite — being so fidgety or restless that you have been moving around a lot more than usual	0	1	2	3
9.	Thoughts that you would be better off dead or of hurting yourself in some way	0	1	2	3

Total Score: 10

Interpretation:

Remember, PHQ-9 is a screening test and does not diagnose. But CAN measure severity and trend response to therapy.

PHQ score ≥ 10 : Likely Major Depression

5-9: Mild Depression

10-14: Moderate

15-19: Moderately Severe

≥ 20 : Severe

BACK

2022–2025 Update — Screening & Newer Agents

USPSTF 2023 (final): screen ALL adults for DEPRESSION (Grade B) and adults <65 for ANXIETY disorders (Grade B); both include pregnant/postpartum. Insufficient evidence to screen for anxiety at ≥65 (I). No routine suicide-risk screening in asymptomatic adults (I).

Antidepressant discontinuation: ANTLER trial (Lewis, NEJM 2021) — stopping after long-term use → ~56% relapse vs ~39% maintained. Taper slowly; withdrawal can be prolonged.

Newer oral agents (context, not first-line for routine GAD/MDD):

- Dextromethorphan–bupropion (Auvelity) — FDA Aug 2022; first rapid-acting oral NMDA-antagonist for MDD (separation from placebo by week 1).
- Zuranolone (Zurzuvae) — FDA Aug 2023; first ORAL agent for POSTPARTUM depression (14-day course); not approved for general MDD.
- Esketamine (Spravato) intranasal — treatment-resistant MDD; expanded to monotherapy 2025.

Standing teaching (PHQ-9/GAD-7, SRI first-line, close early follow-up, side-effect-guided selection) verified CURRENT — no change.

Take-Home Points

PHQ-9 and GAD-7 are the preferred tools to DIAGNOSE and MONITOR unipolar depression and GAD; PHQ-2/PHQ-4 are screens only — don't use them when pretest probability is high.

Short-term follow-up (within ~1–2 weeks) after starting a drug is critical — watch for emergent suicidality/mania, intolerable side effects, and non-adherence.

SRIs are first-line for both; pick the agent by comorbidity, prior response, cost, preference, and SIDE-EFFECT profile (use side effects to your advantage).

Treat the somatic symptoms too (e.g., PT, headache care) and keep re-screening for bipolar disorder and substance use.

USPSTF 2023: screen all adults for depression and adults <65 for anxiety.

Attribution & Optimization Notes

Adapted from TeachIM.org — “Depression and Anxiety” (Trefilek, Melton, Guinn, Fainstad; Section Ed. Brett; Univ. of Colorado). Published Dec 2021, updated Nov 2022.

Source URL: https://teachim.org/teaching_material/depression-and-anxiety/

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Optimized June 2026: original interactive board preserved (all 18 slides, animations & BioRender graphics intact); no factual/typo errors found → append-only edit (this 3-slide addendum).

Currency verified vs USPSTF 2023 screening recommendations, ANTLER (NEJM 2021), and FDA approvals (Auvelity 2022, Zurzuvae/zuranolone 2023). Companion: Depression-learner-handout.docx.

Educational use only — clinician-reviewed before teaching. Suicide risk is a sensitive topic; ensure local crisis resources are available.